

An Open Letter to Healthcare Professionals

I'm a scientist by training, though not in clinical medicine. My work revolves around **research, mechanistic modeling, and publishing in peer-reviewed journals**. Because of my background, I'm comfortable with **mechanistic extrapolation**: understanding how a system works, imagining a change, and predicting how the system will respond. When you're careful and thorough, **you're often, but not always, right**.

But that's not how the FDA approves drugs.

Regulatory approval requires statistically demonstrated safety and efficacy across large, expensive, multi-phase clinical trials. This system insures that any drug gaining FDA approval is safe and effective. But the system has a huge failure: many drugs that are safe and effective never get a change to prove themselves in trials

- If a drug doesn't promise enough profit, the trials don't get run
- If the intellectual property doesn't support exclusivity, the trials don't get run.
- If a compound improves **healthspan or physiological resilience** rather than treating a specific disease, the trials don't get run either, because FDA endpoints must be disease-specific. **"Making healthy people healthier" is not an approvable indication.**

This leaves us with a growing category of **peptides, small molecules, and biologics** supported by strong mechanistic evidence, promising preclinical data, and often early human signals, but **no Phase III trials**. That reality leads people like me to conduct **careful, informed N=1 research** on ourselves.

I take that responsibility seriously. This is my version of informed consent: before I inject anything subcutaneously, I do a **deep dive into the published literature** of in vitro studies, animal models, early human trials, and mechanistic reviews. I look for **short-term safety signals**; if it's not there, it's a no-go. The lack of PK data is always a challenge, so dosing requires **caution, conservatism, and iteration**. But with enough research, you can build a reasonable picture of how a peptide behaves biologically.

Here's what that looks like for me:

- **Retatrutide for metabolic stability** — my CGM data shows profound improvements. GLP-1s should be inexpensive and widely available to everyone approaching prediabetic ranges.
- **Tesamorelin + Ipamorelin every morning, Ipamorelin + CJC-1295 NODAC every evening**, cycled 12-weeks ON and 6-weeks OFF to prevent receptor desensitization and

pathway adaptation. They **reduce visceral fat, improved sleep, enhanced tissue repair, and support body recomposition**. My labs reflect these changes, I feel the functional difference, and I miss those benefits during off-cycles.

- **Thymulin and Thymosin α 1 twice weekly** for immune modulation. It's hard to "feel" immune improvement, but **Thymulin resolved chronic overnight post-nasal drip** I'd had for years. If Thymulin can modulate my NF-kB signals to reduce proinflammatory cycles there, I'm betting it is helping inflammaging throughout my body.
- **GHK-Cu daily** for tissue repair, and **KPV daily** for inflammaging.
- **TB-500 and BPC-157**, cycled 2 weeks on / 2 weeks off, produce **noticeable recovery benefits** after hard workouts or heavy yardwork.
- **ARA-290 in a staggered weekly pattern (4/2/1 mg M/W/F)** for **small-fiber nerve protection, microvascular resilience, and systemic inflammatory modulation**.
- And although they're not peptides, I also take **metformin and tadalafil off-label** because the **peer-reviewed healthspan literature is extraordinary**. The real question isn't why I'm using them, it's why these inexpensive, well-studied treatments **aren't standard of care for aging populations**.

Some peptides carry real risks, for example **MT-II has clear cancer concerns**. Some peptides, like **GH secretagogues**, **require periodic labs to adjust dosing**. Other peptides, like **TB-500 are strongly angiogenic and warrant cancer-screening conversations**. Others have **excellent safety profiles but mixed efficacy**, like **DSIP**. And some, like **SS-31, remain popular despite failing Phase III endpoints**.

Some of your patients are already using BPC-157, TB-500, GHK-Cu, KPV, Retatrutide and other peptides. And more will be using them soon. Some are **thoughtful, careful, and research-driven**. Others may be inexperienced and recklessly relying on gym-bro advice and Reddit threads. You and your patients need an **informed partnership**: The labs and treatments you recommend need to take into consideration what they have chosen to do.

This partnership requires them to feel **safe telling you everything they're doing**. And they won't tell you if they expect to be lectured. It's perfectly appropriate to say: **"These are not FDA-approved for human use. I cannot recommend them. We don't have sufficient clinical data to know they're safe or effective."** But it's equally important to add: **"Thank you for telling me. I'm not judging you. I want to take this into account as I care for you."** That simple acknowledgment opens the door to **safer, more informed care**.

And while you don't need to become peptide experts, **some awareness makes you better clinicians**. You don't need hours to catch up; you can ask an AI research assistant to

summarize the clinical and preclinical literature on BPC-157, TB-500, GHK-Cu, KPV, or any other peptide. **You'll be surprised how quickly you can get oriented.**

If a patient is using **GH secretagogues like Tesamorelin, Ipamorelin, or CJC-1295**, you need to know they need **IGF-1** in their annual labs. If they're using **angiogenic peptides, like TB-500**, you need to discuss **cancer risk and screening**. If they mention "**Glow blend**," you need to know it contains **TB-500**.

I have a model relationship with my PCP. I inform her about everything. **All my peptides are logged in my EHR.** We discuss what additional labs are appropriate. She never endorses my choices, but she doesn't scold me either.

My hope is that healthcare professionals stay **informed enough to help patients navigate these choices safely, compassionately, and without judgment.** The landscape is changing. Patients are experimenting. The **mechanistic, preclinical, and early clinical evidence** strongly suggests that certain peptides may offer **meaningful health benefits when used responsibly.**

Thank you for taking the time to understand this emerging space. Your willingness to stay informed makes a profound difference in patient safety and trust.

With Respect and Appreciation,
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